

1. Is the NHS keeping up with patient demand?
2. How are GP practices adapting?
3. What can we learn from external data?



Two words of caution:

- unprecedented global demand on healthcare
- incomplete data

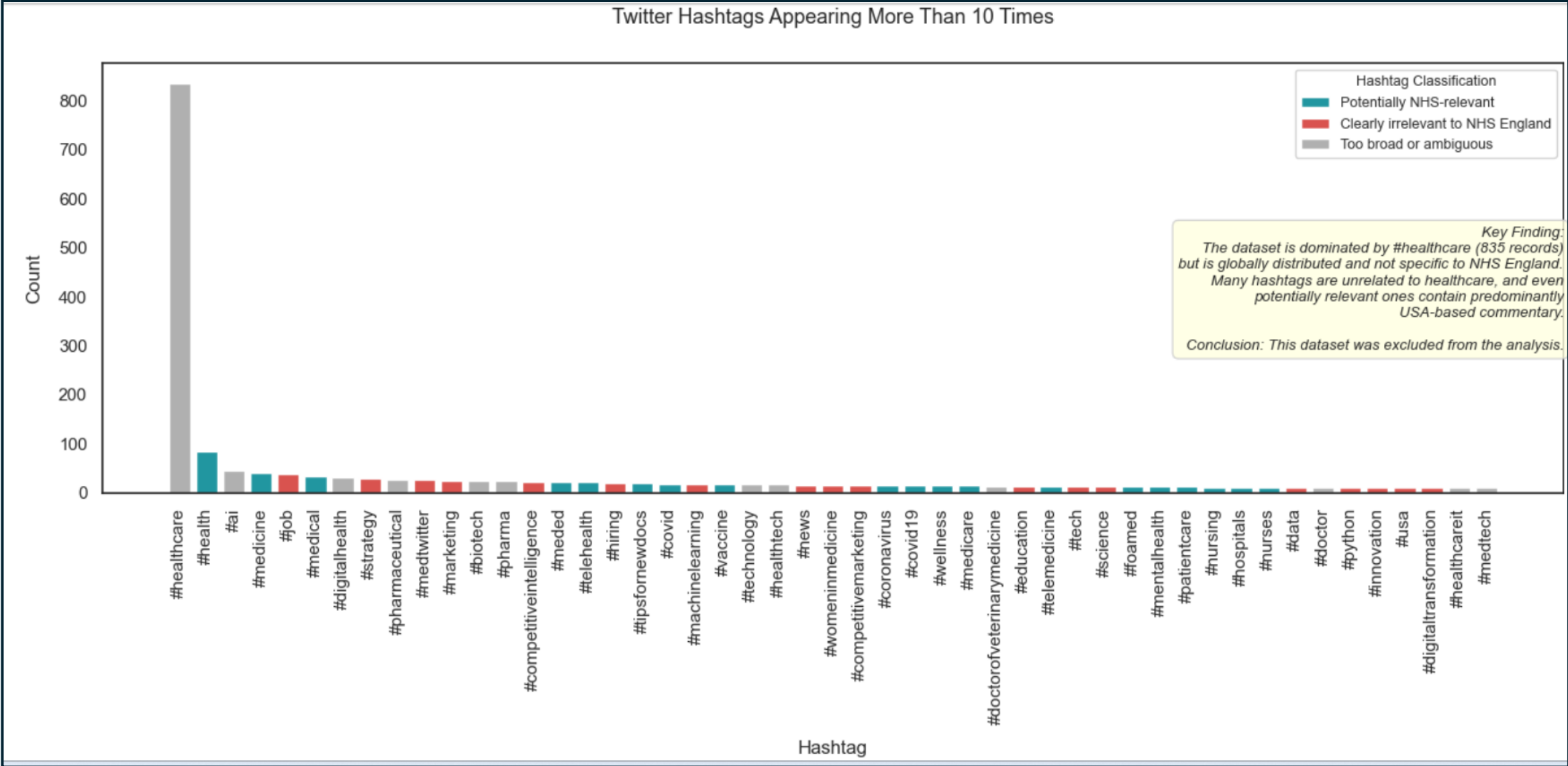
Key analytical questions



1. Has there been adequate staff and capacity in the networks?
2. How are GP practices responding to demand pressure?
3. What can external data tell us that appointment records cannot?



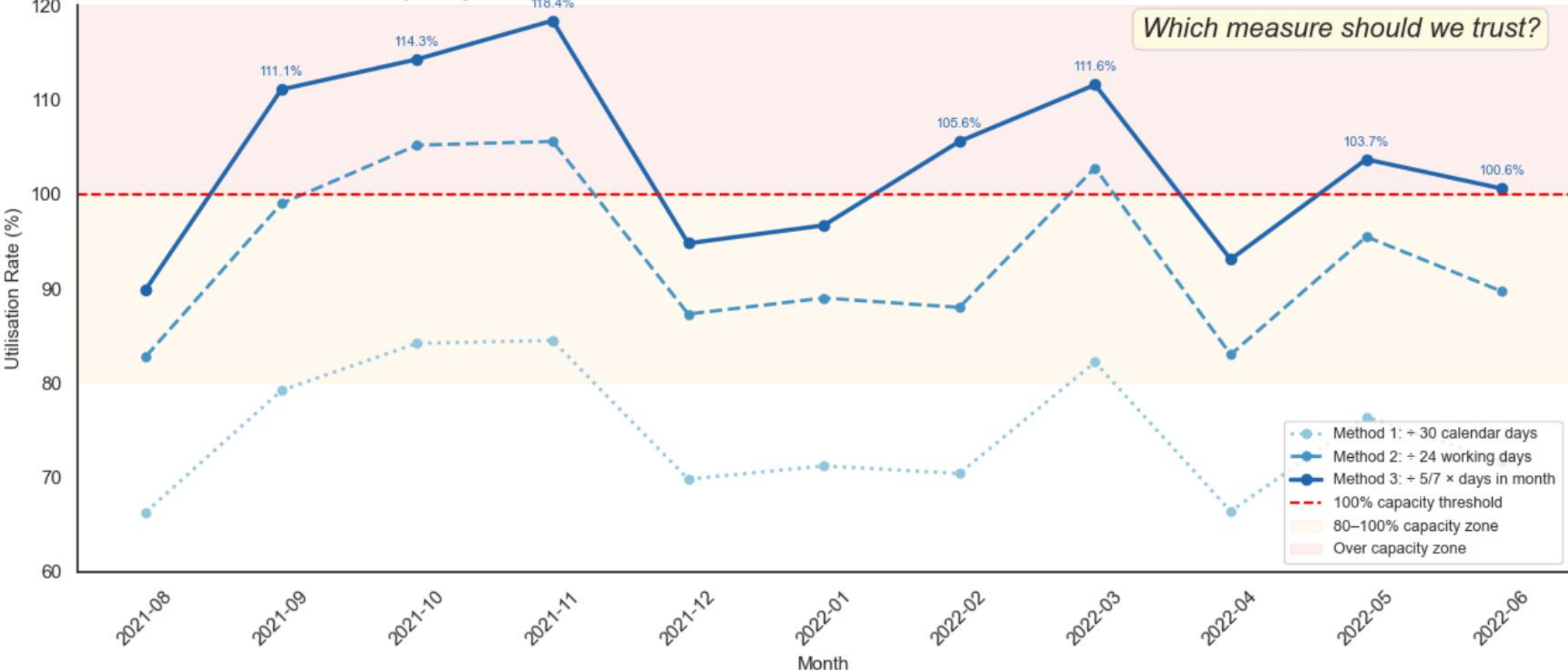
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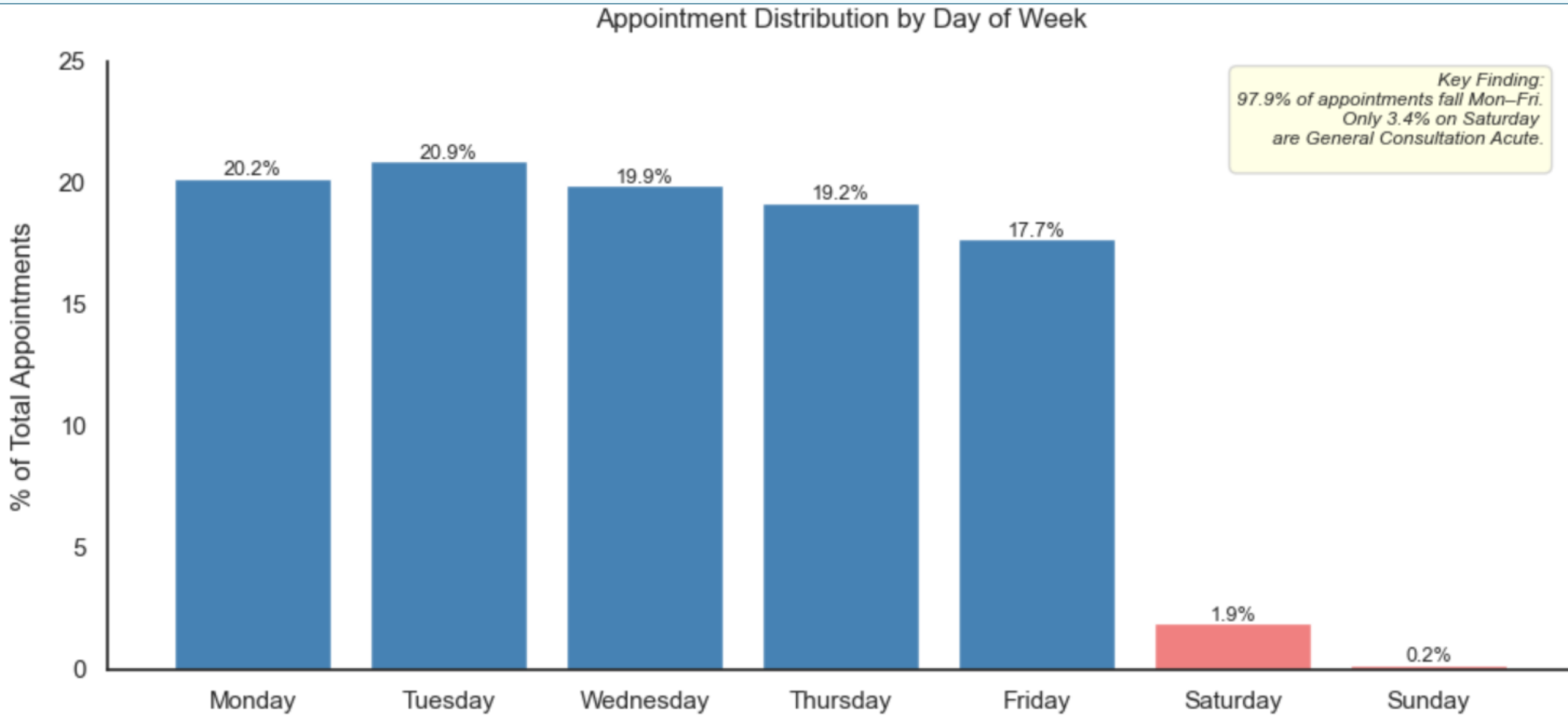
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3 Measures of National Capacity Utilisation



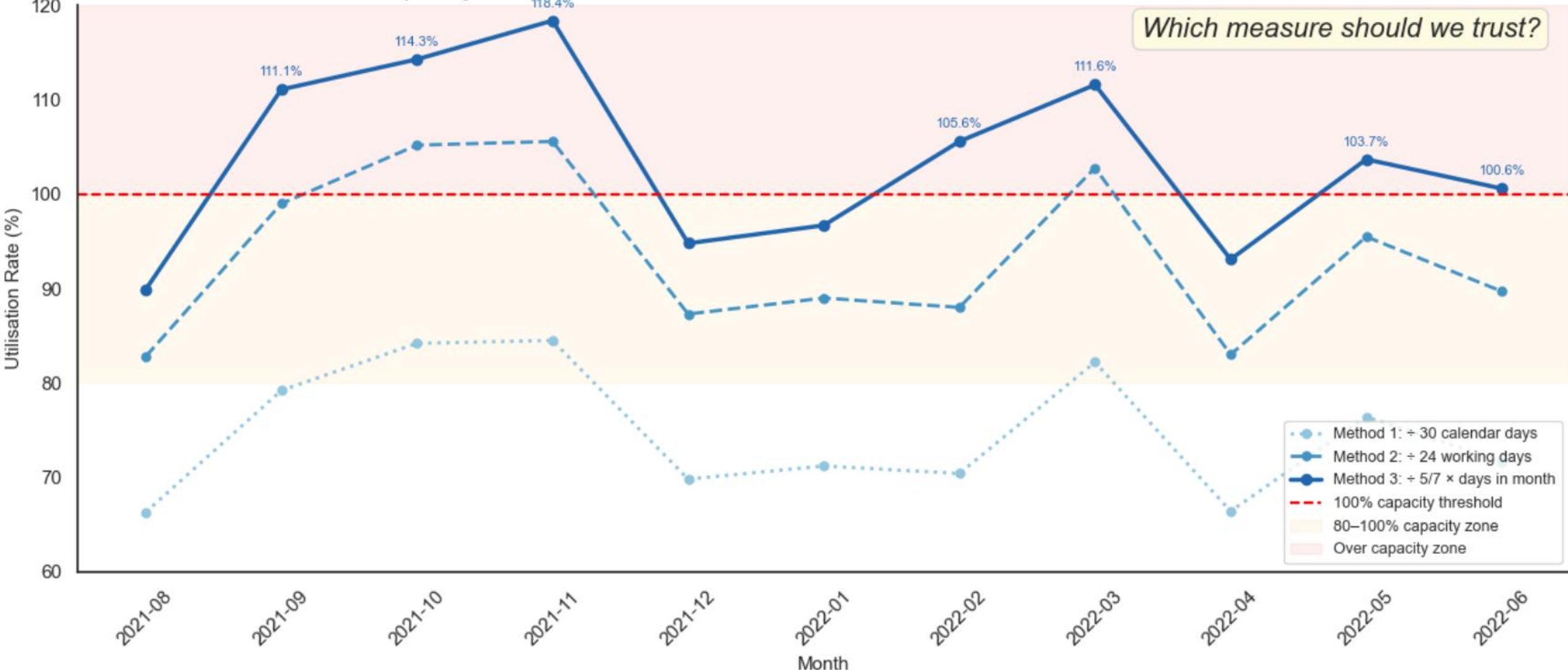
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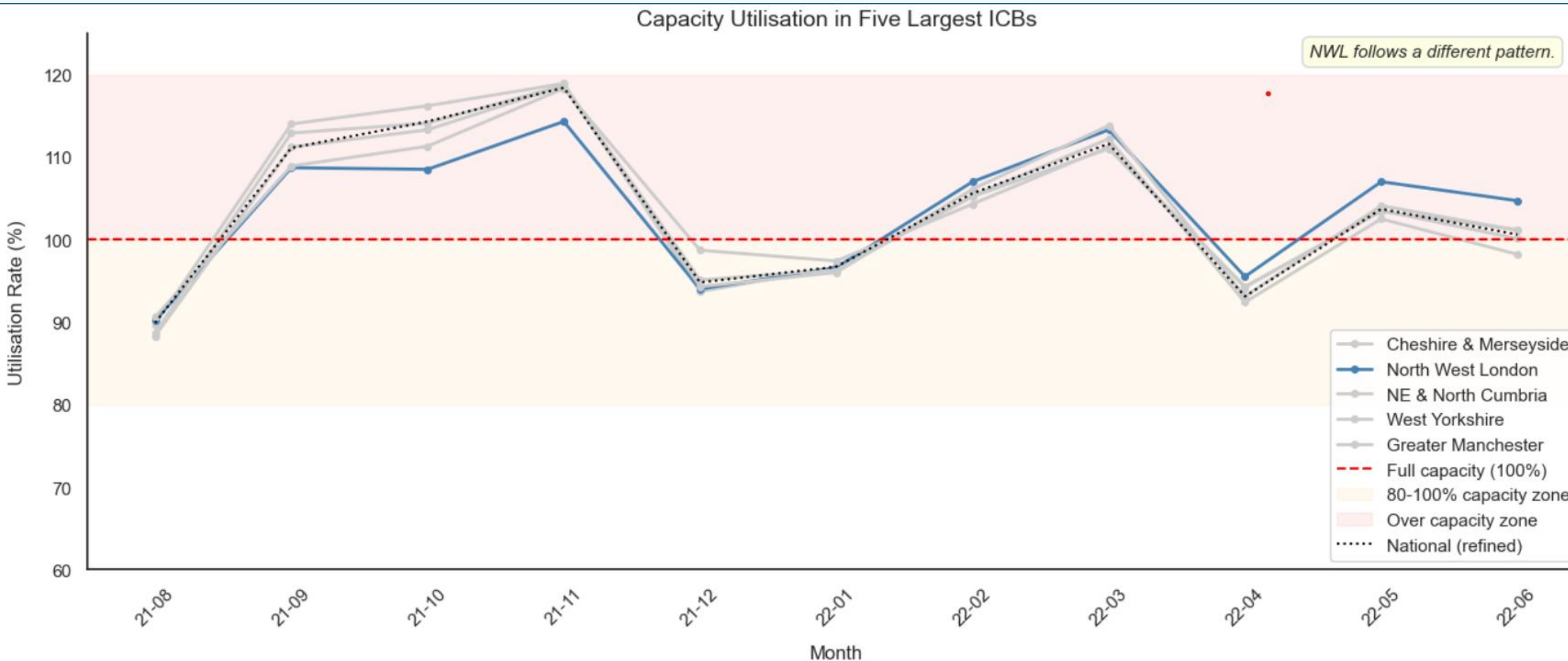
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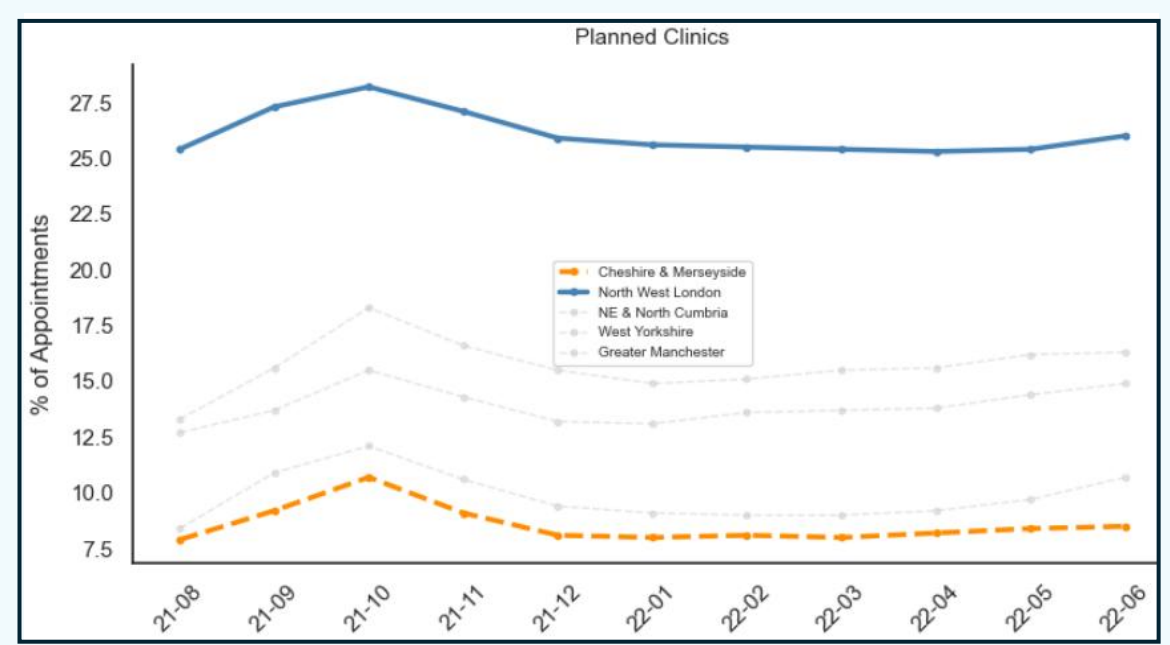
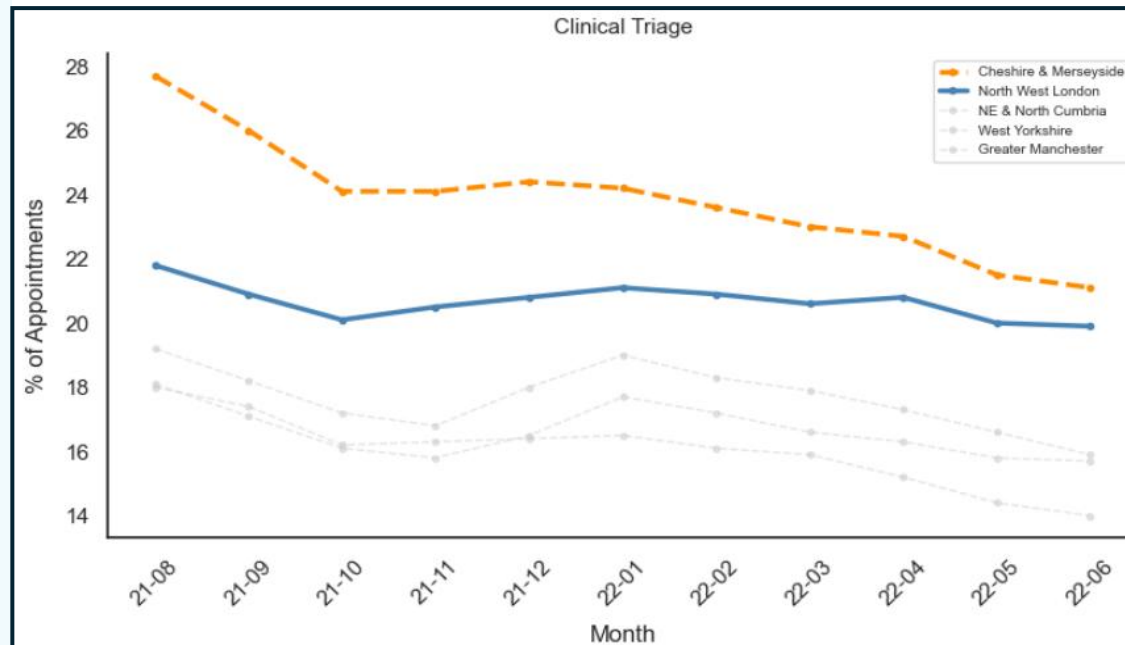
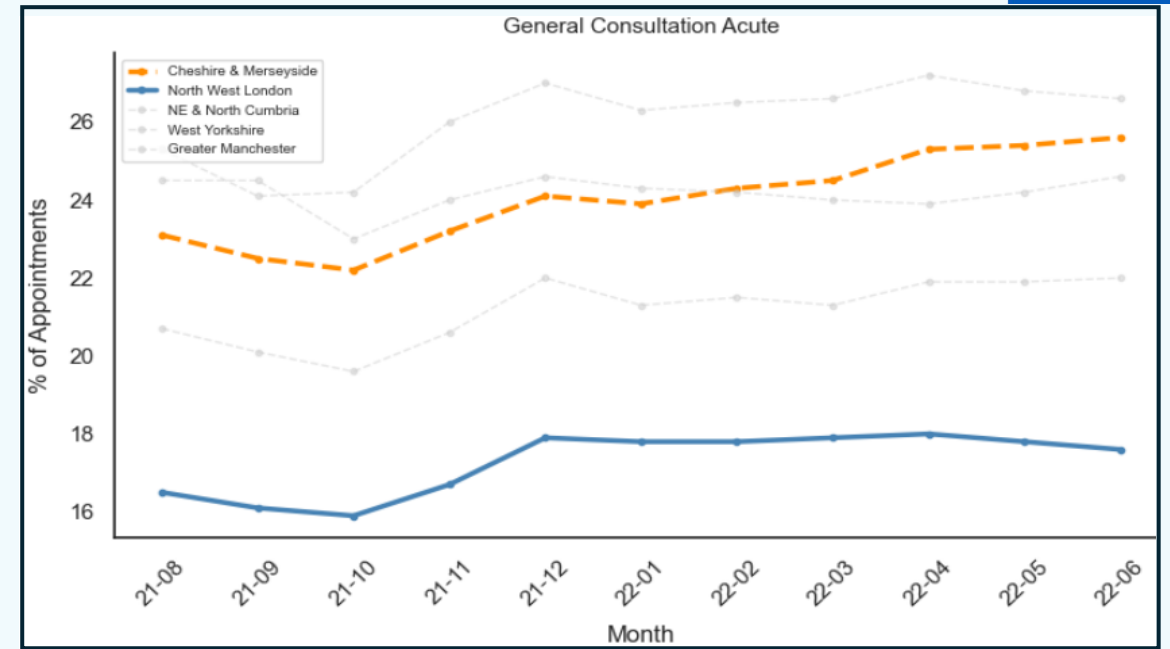
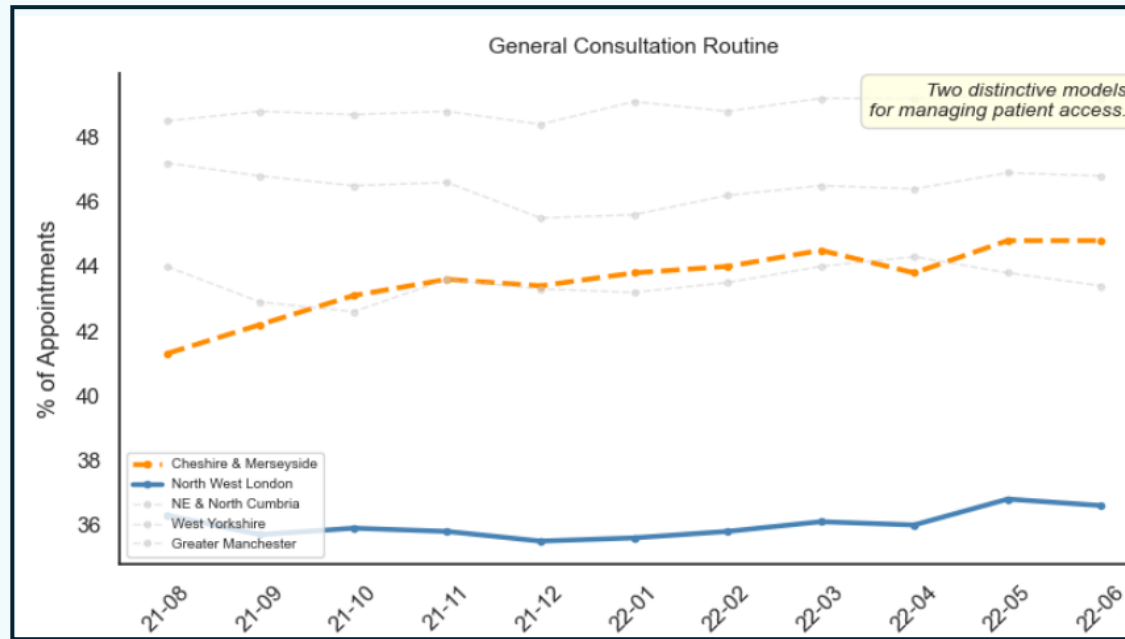
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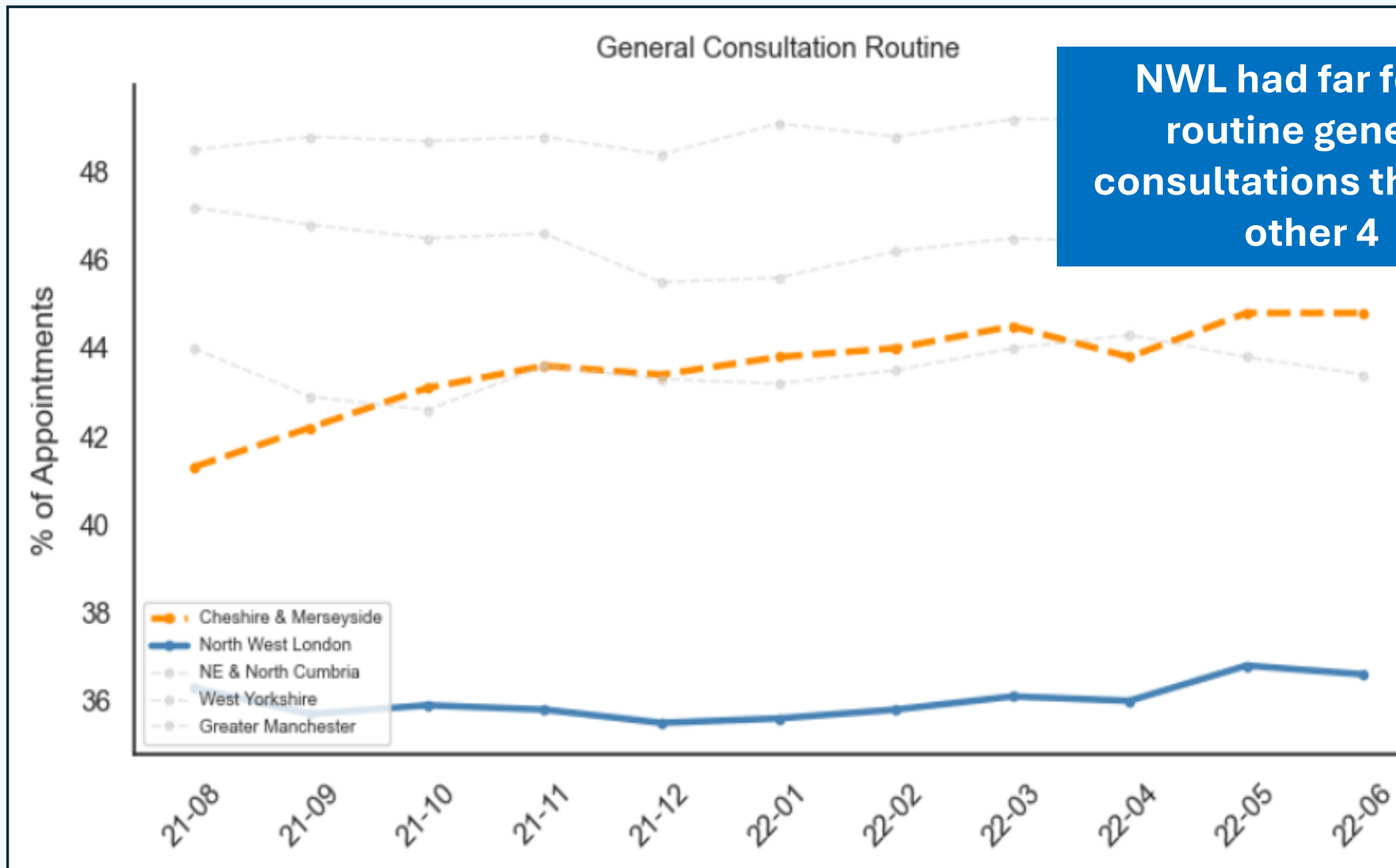
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How are GP practices responding to demand pressure? NWL vs C&M

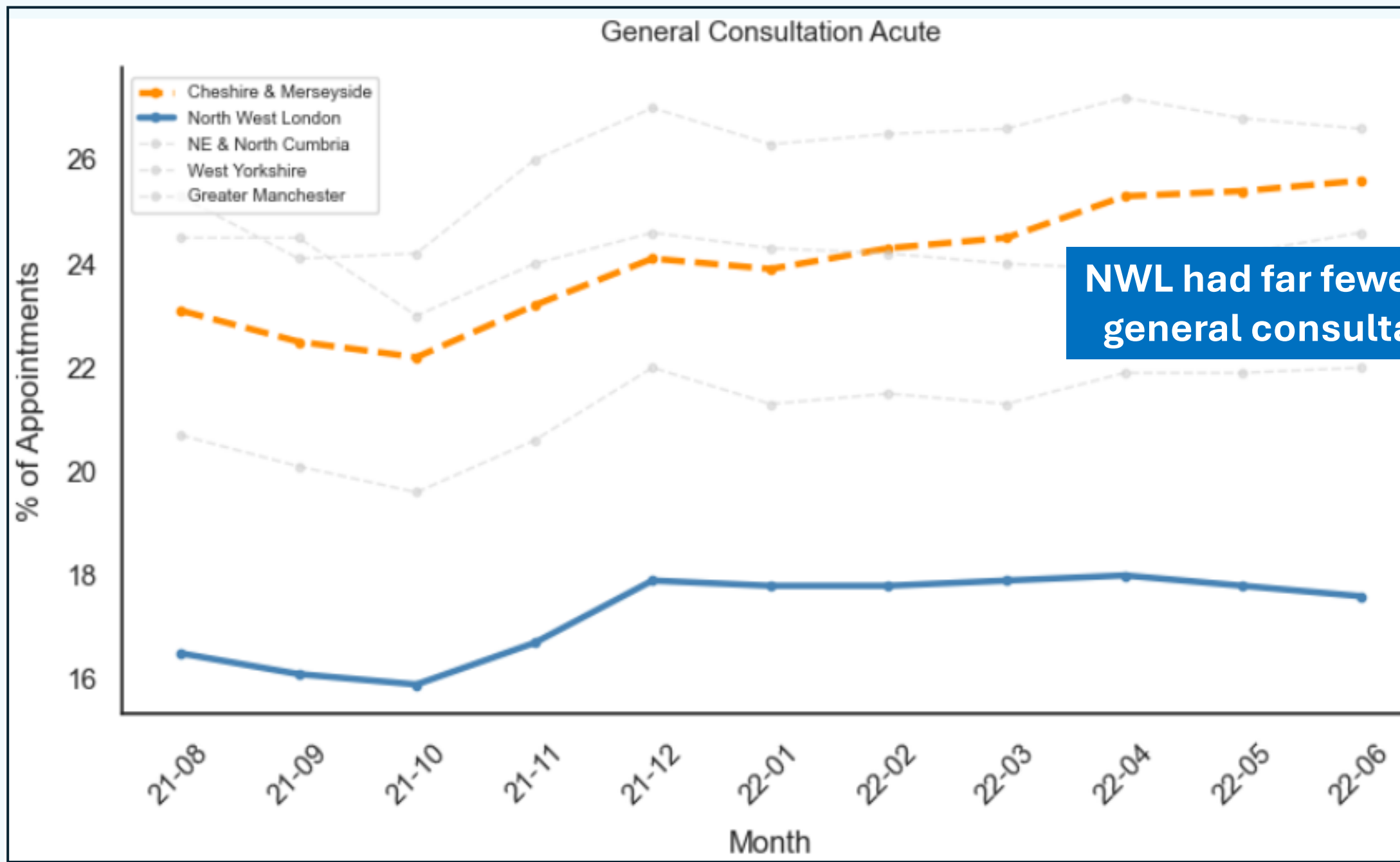


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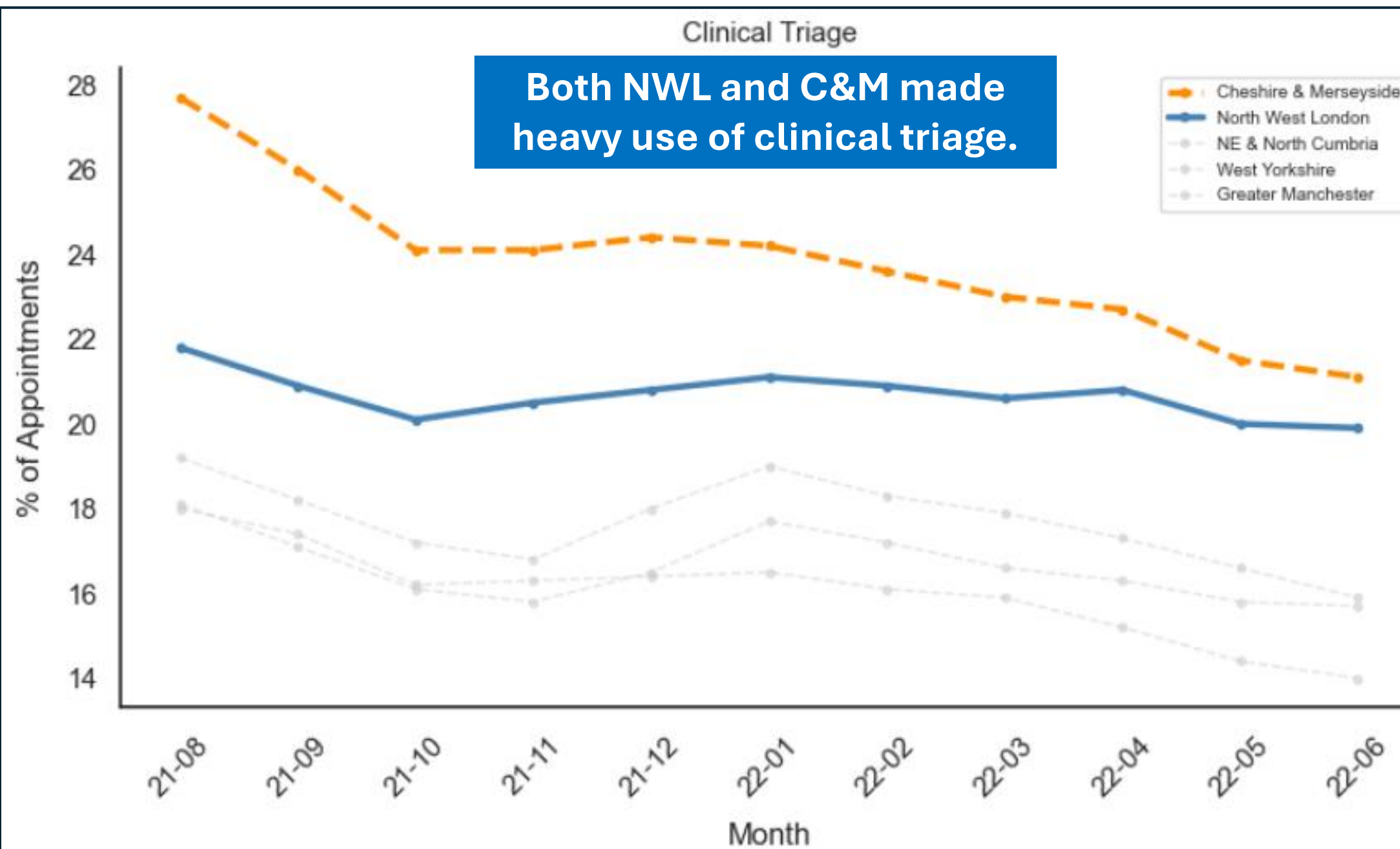
NWL had far fewer routine general consultations than the other 4

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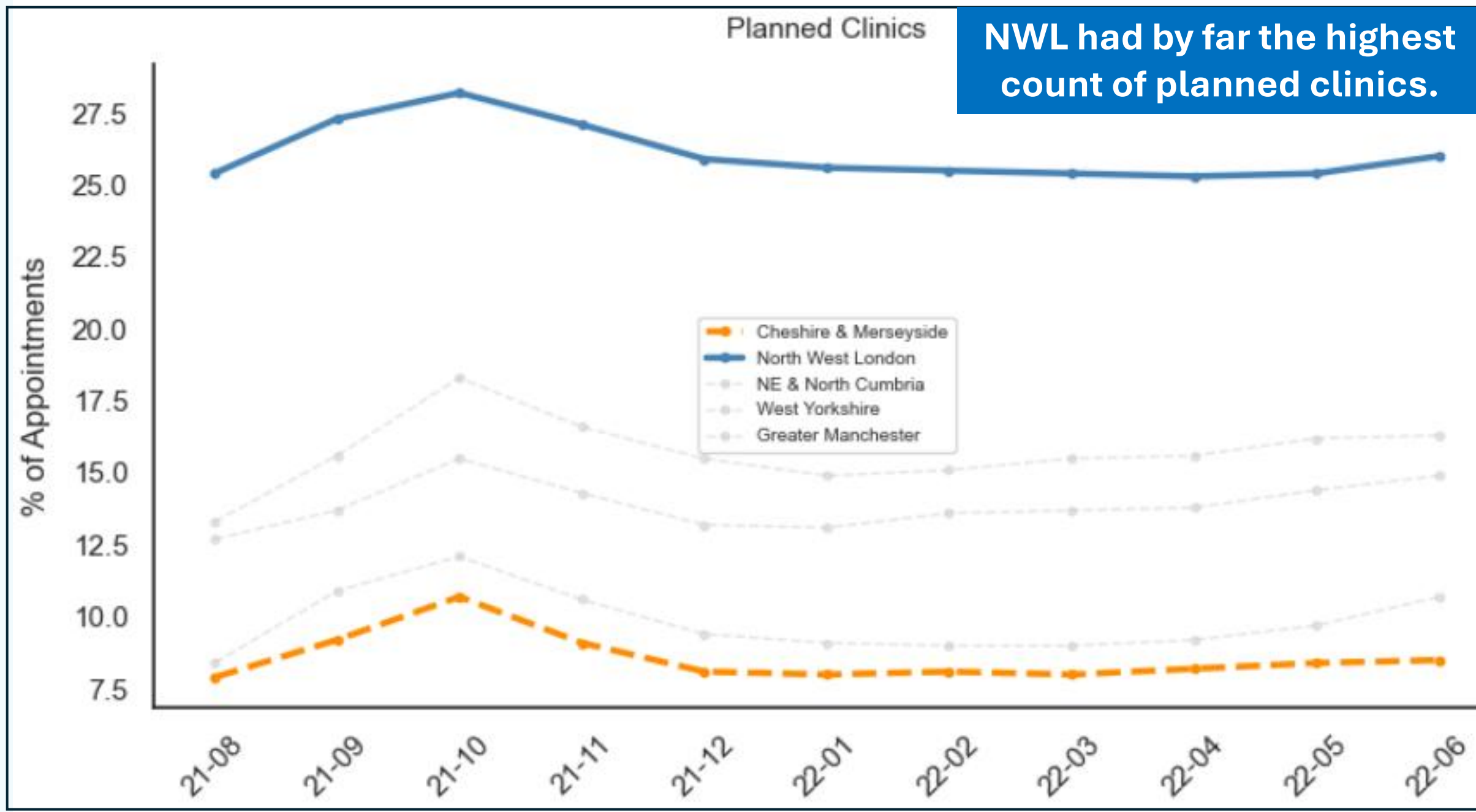


NWL had far fewer acute general consultations.

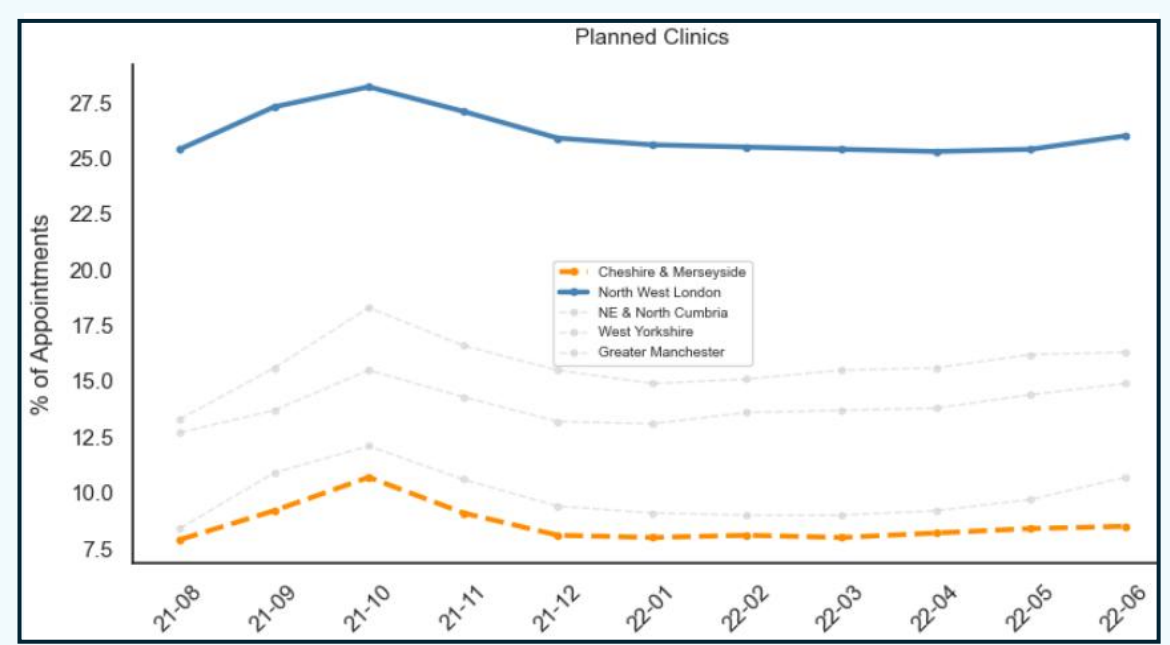
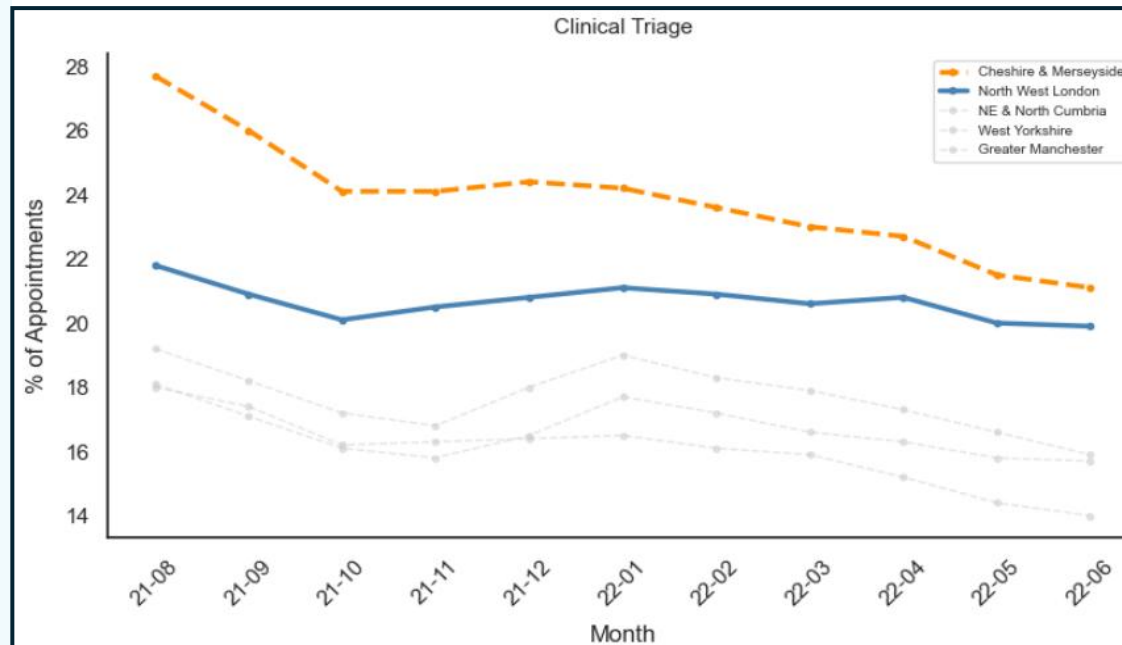
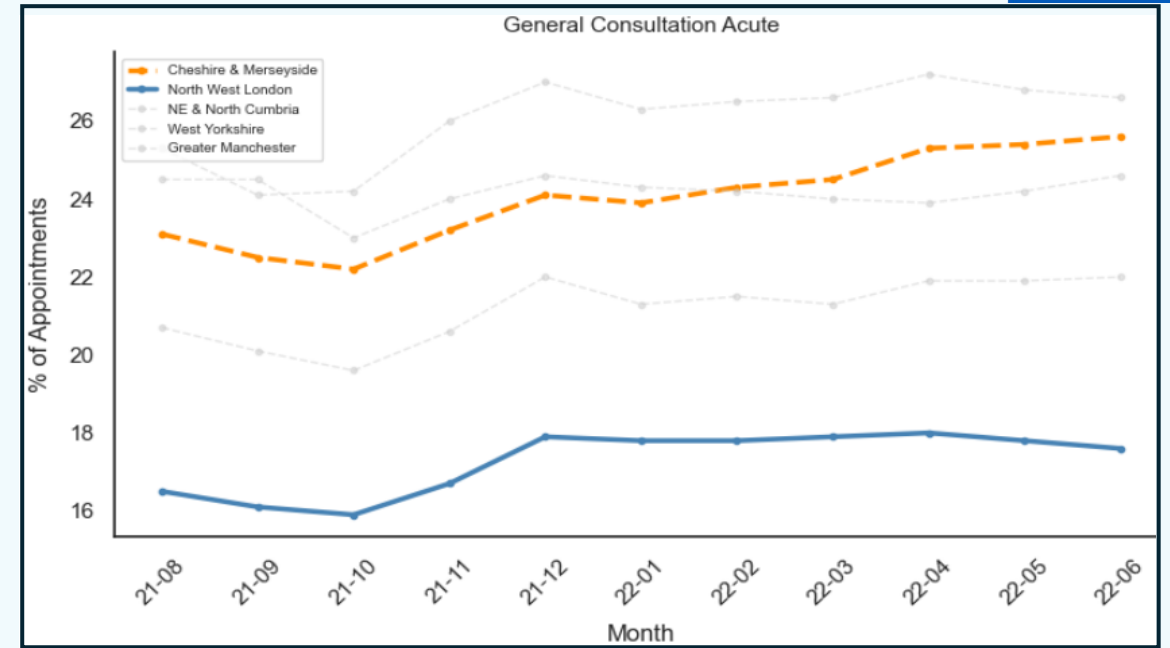
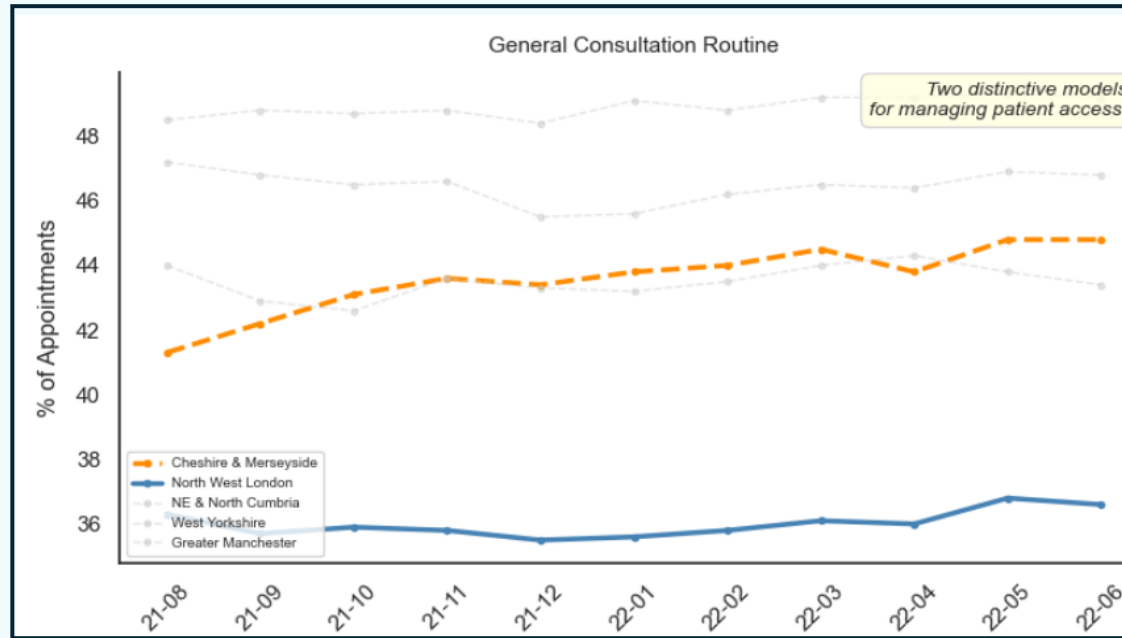
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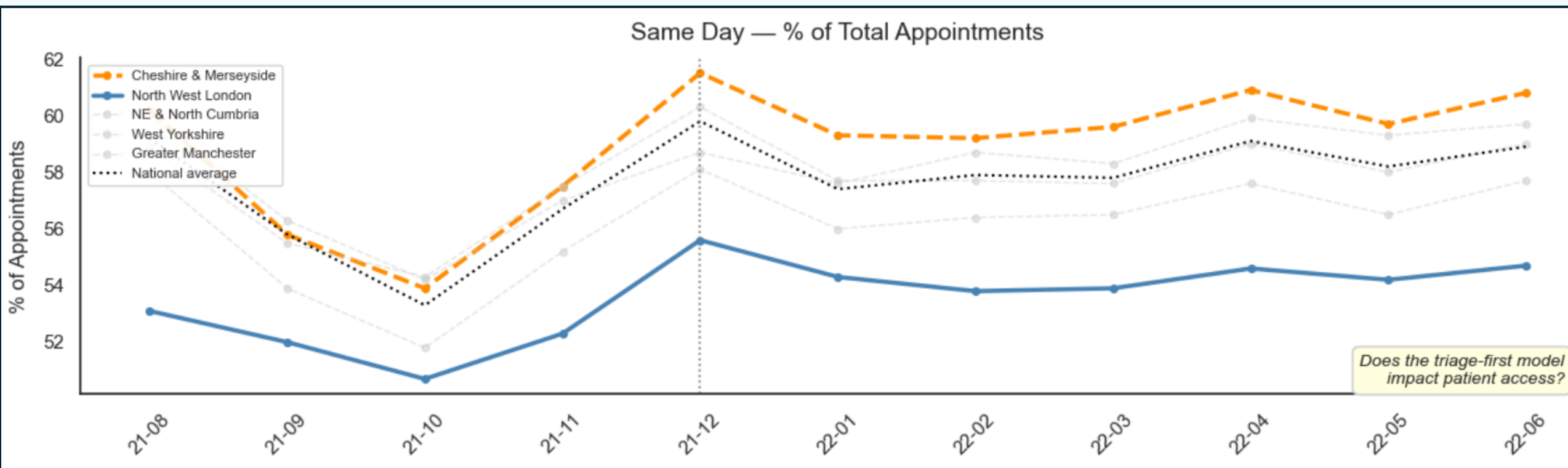
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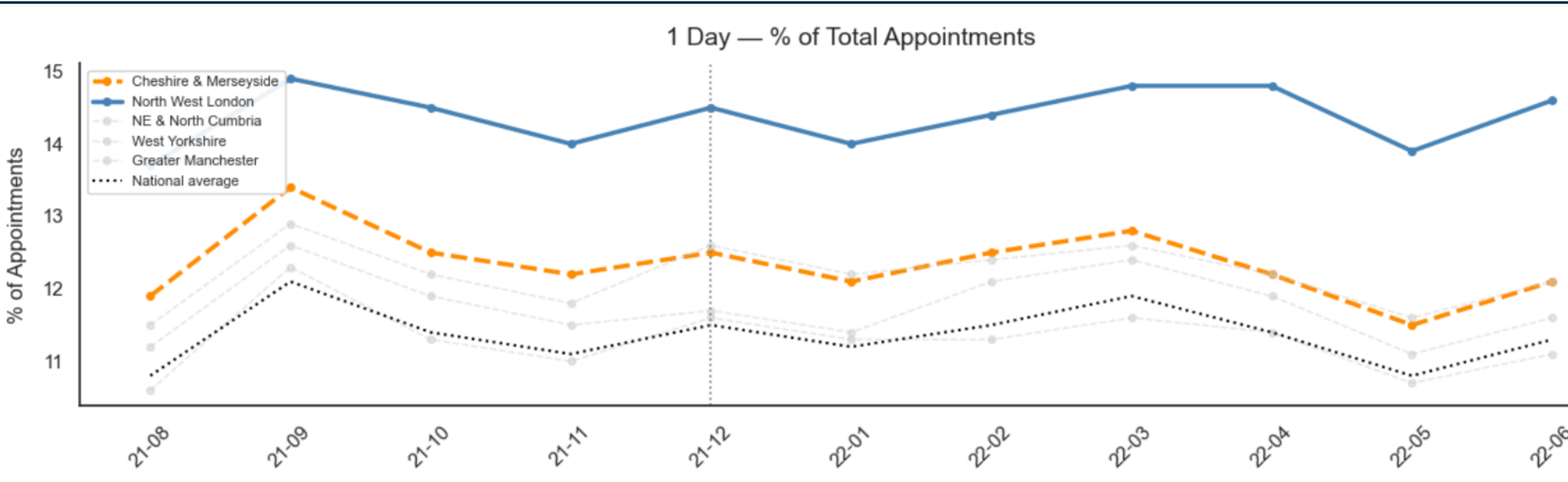


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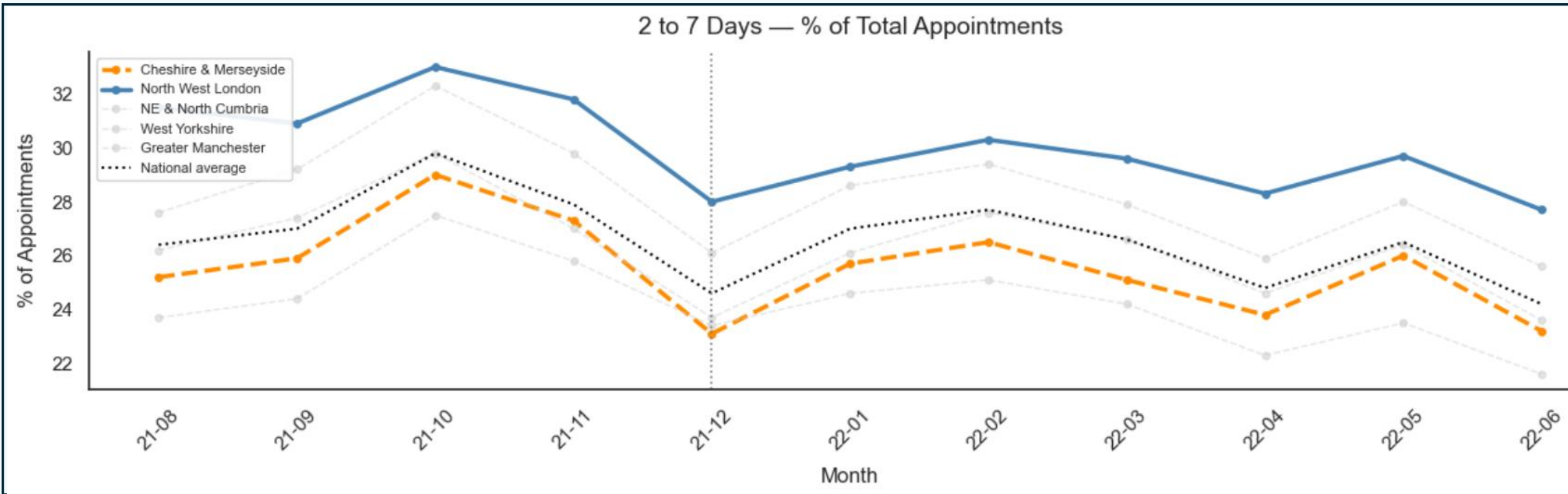
**C&M offered the highest percentage of same day appointments.
NWL offered the least.**

How are GP practices responding to demand pressure? NWL vs C&M



NWL offered the highest percentage of appointments with a 1 day lead-in time.

How are GP practices responding to demand pressure? NWL vs C&M



NWL offered the highest percentage of appointments with a 2-7 day lead-in time.

The unanswered question:

Should NWL's triage and planned clinic
model be rolled out across England?

RECOMMENDATIONS

1. Investigate NWL's access performance before recommending triage model

- clinical outcomes
- A&E attendance rates

2. Gather patient feedback

- access
- reasons for DNAs

3. Introduce more granular lead-time bands

- current 2-7 day band cannot support clinical safety monitoring